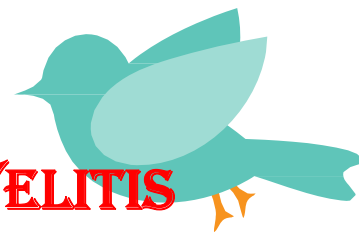


EXAMINATION OF CASE OF OSTEOMYELITIS



CHIEF COMPLAINTS:-1.PAIN

2. DISCHARGING SINUS

3. DEFORMITY

HISTORY /PRESENTING ILLNESS:- Elaborate on pain (SOCRATES), Deformity(onset, duration, increasing) About sinus (is it active-means discharging type of discharge,color , bony spicules, duration, aggravating and reliving factors).

ADL:- Is he able to do his routine activites for that particular limb Eg , like walking, cycling, running , using his upper limb for eating food , combing hairs etc.

NEGATIVE HISTORY:- H/O Trauma, Compd #s, Fever, Repeated operations, Implants in situ, Pinprick, painful swelling – Garres and brodies, Foreign body, ear infection , lung infection(Tb) ,chronic debilitating conditions like anemia ,Tb, DM, Diarrhoea(salmonella),constitutional symptoms in TB, Night cries-seen in TB, Sickel cell anemia, osteoid osteoma, STD-syphilis, Native treatment, similar swelling in other sites-CRMO. (Salmonella osteomyelitis seen in sickle cell anemia), malignancy .Drugs-long term ingestion of anti cancer drugs,

LOCAL EXAMINATION

ATTITUDE :- If upper limb- start with shoulder, elbow , wrist , fingers, if its lower limb- start with Asis, hip, knee , ankle and foot,(just say how he keeps it)

DEFORMITY :- FFD, Angular def, bowing etc

INSPECTION :- look **Antrly**, **postrly** and from **sides**.

Antrly- start from ASIS,at same level, look for wasting of muscles(Q-ceps), look for parapatellar fullness, effusion (reactive), look for bowing and angulation in the diaphysis,

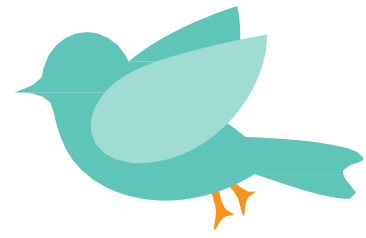
Describe the sinuses- site, size-say 1cm, discharge-pus, color of discharge , is it copious, any bony spicules visible , is it healed, puckering of skin , hyper pigmentation, scars healed by primary or sec intention. Look for above points **postrly** and from **sides**

PALPATION:- Local rise in Temperature , Tenderness,

Superficial :- painful in cellulitis

Deep:- Muscles-look for wasting of muscles, induration, fibrosis,





BONY palpation;- Palpate along the entire bone and look for (TTIACT);-

THICKENING,

TENDERNESS,

IRREGULARITY,

ABN MOBILITY,

CREPITUS,

TRANSMITTED MOVEMENTS ,

SINUSES;- Palpate for the sinuses Look for- Induration, Adherence of sinus to the underlying bone , Squeeze –check for discharge-pus, note color(green –pseudomonas, black-fungal, sandy-TB,, bloody, is it copious,and bony spicules.

ULCER;-note size, shape, surface , margin, floor, fixity to underlying structures.

MOVEMENTS ;;- Look for ACTIVE& PASSIVE ROM , FFDs, contractures–IN the Joint above and joint below

MEASUREMENTS;- LINEAR – True length and Segmental length discrepancy(usually there will be lengthening in osteomyelitis in children)

CIRCUMFERENTIAL :- Thigh and calf muscle wasting

DIAGNOSIS;- Anatomical – which bone is involved, site metaphyseal , Diaphyseal.

Pathological –chronic osteomyelitis.

INVESTIGATION ;;-

LABORATORY;-Hb , wbc, TcDc,ESR,CRP,Blood culture.

RADIOLOGICAL: -plain x ray, AP & LAT views

SPECIAL INVESTIGATIONS;- CT (sequestrum), MRI, Technitium Scan ,



SELF NOTES

